# Structural Conflicts in Washington State Nursing CBAs

A Theory of Constraints Analysis

Research Report

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December 2025

Version 1.0

**Request for Feedback**

This report presents findings from an analysis of six collective bargaining agreements. The author welcomes corrections, alternative interpretations, and suggestions for extending the analysis. If you see errors in the methodology or conclusions not supported by the evidence, please reach out.

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## Executive Summary

Hospitals across Washington State face persistent nursing workforce challenges. Vacancy rates, turnover, and competition for experienced nurses strain operations and affect patient care. While wage competition receives significant attention, less visible factors may also influence nurse retention and workplace satisfaction.

This report presents findings from a structured analysis of ten collective bargaining agreements (CBAs) between Washington State health systems and the Washington State Nurses Association (WSNA). Using Theory of Constraints methodology, the analysis identified "embedded conflicts" within these agreements: provisions that create genuine dilemmas where nurses or managers must choose between two legitimate but incompatible requirements. Across the ten agreements, 134 distinct conflicts were identified and analyzed.

The analysis found four conflicts present in all ten agreements examined, suggesting these are structural features of the hospital-nurse employment relationship rather than artifacts of any particular negotiation. These universal conflicts involve meal and rest break rights versus continuous patient care, low census procedures versus income security, floating requirements versus competency protections, and schedule stability versus operational flexibility.

The presence of these conflicts across all agreements suggests an opportunity. A health system that developed effective resolution mechanisms for even one of these universal conflicts could differentiate itself in a competitive labor market. Rather than competing solely on wages, such a system would offer something harder to replicate: a workplace where fundamental dilemmas have been addressed rather than papered over.

This report describes the methodology, presents the findings across all ten health systems, and discusses implications for workforce strategy.

# 1 Introduction

## 1.1 Background

Washington State hospitals operate in a challenging labor environment. The state's nursing workforce has been affected by the same forces shaping healthcare nationally: pandemic-driven burnout, an aging workforce, regional competition for talent, and evolving expectations about work-life balance. Legislative changes, including Senate Bill 5236 mandating staffing plan compliance,[1] have added regulatory complexity to operational challenges.

In this environment, health systems compete intensively for nursing talent. Wage competition is visible and well-documented. Less visible are the day-to-day working conditions shaped by collective bargaining agreements: how schedules are managed, how low census is distributed, how floating decisions are made, how breaks are protected.

These operational details matter. A nurse considering employment options weighs more than salary. Predictability, fairness, autonomy, and respect all factor into the decision. These qualities are difficult to measure but are shaped, in part, by the rules governing the employment relationship.

## 1.2 Research Question

This analysis asked a specific question: Do collective bargaining agreements in Washington State contain structural conflicts that place nurses and managers in "rock and a hard place" situations? If so, are these conflicts unique to particular agreements, or do common patterns appear across the industry?

The hypothesis was that such conflicts would exist and would cluster around predictable themes: the tension between operational flexibility and worker protections, between cost control and fair treatment, between patient care demands and legally mandated rights.

## 1.3 Methodology

The analysis applied Theory of Constraints (TOC) conflict analysis to ten collective bargaining agreements between Washington State health systems and WSNA.[2] TOC, developed by Dr. Eliyahu Goldratt,[3] provides a structured method for identifying conflicts where two legitimate requirements cannot both be satisfied simultaneously.

For each potential conflict identified, the analysis asked:

- What is the common goal both parties share?
- What are the two legitimate requirements in tension?
- What specific provisions create the conflict?
- What assumptions underlie the conflict?

[1] Washington State Senate Bill 5236, "Concerning hospital staffing standards," enacted 2023. Codified primarily in RCW 70.41. The law requires hospitals to develop staffing plans with nurse input and comply with those plans, with enforcement beginning in 2025.

[2] All agreements analyzed are between facilities and the Washington State Nurses Association (WSNA). Agreement texts were obtained from WSNA's public website at `https://www.wsna.org/union` and from health system publications.

[3] Goldratt, Eliyahu M. *The Goal: A Process of Ongoing Improvement*. North River Press, 1984. The "Evaporating Cloud" conflict resolution tool used in this analysis is described in Goldratt's subsequent work *It's Not Luck* (1994) and formalized in the Theory of Constraints body of knowledge.

- How does this conflict manifest in daily operations?

The ten health systems analyzed were selected to represent geographic, organizational, and size diversity within Washington State:

| **Health System** | **Type** | **Conflicts** |
|---|---|---|
| EvergreenHealth | Large suburban | 18 |
| Ocean Beach Hospital | Small rural | 16 |
| Grays Harbor Community Hospital | Small rural | 15 |
| Skagit Regional Health | Mid-size regional | 14 |
| Cascade Medical Center | Critical access | 14 |
| Providence Holy Family Hospital | Mid-size urban | 13 |
| Overlake Hospital Medical Center | Large suburban | 12 |
| Whidbey Health Medical Center | Small rural | 12 |
| Seattle Children's | Specialty pediatric | 11 |
| Providence Sacred Heart | Large urban | 11 |

Table 1: Health Systems Analyzed (134 Total Conflicts Identified)

The variation in conflict counts reflects differences in agreement complexity, scope, and specificity rather than quality of labor relations. Larger agreements with more detailed provisions, and smaller facilities with unique operational constraints, both create opportunities for conflicting requirements to emerge.

# 2 Universal Conflicts

Four conflicts appeared in all ten agreements analyzed. These represent fundamental tensions in the hospital-nurse employment relationship that transcend any particular negotiation or organizational context.

## 2.1 Conflict 1: Meal/Rest Break Rights vs. Continuous Patient Care

Washington State law[4] and all ten agreements guarantee nurses meal and rest breaks. Patient care, however, is continuous. The conflict arises because no agreement provides a robust operational mechanism to ensure breaks can actually be taken without compromising patient care.

| Health System | Manifestation |
|---|---|
| EvergreenHealth | Break relief role has "emergent" exception |
| Overlake | Break rights stated vs. care continuity assumed |
| Seattle Children's | Uninterrupted breaks promised vs. staffing volatility |
| Providence Sacred Heart | Missed breaks trigger overtime pay (cost dilemma) |
| Providence Holy Family | "Free to leave premises" vs. no coverage mechanism |
| Ocean Beach | Implicit in staffing structure |
| Whidbey | No operational mechanism for break coverage |
| Skagit Regional | "Endeavor to relieve later to avoid overtime" |
| Grays Harbor | Break compliance vs. coverage vs. avoid overtime triangle |
| Cascade | CRN responsible for breaks but may be understaffed |

Table 2: Break Coverage Conflict Manifestations[5]

The core dilemma: agreements state that nurses are entitled to breaks, but do not staff units with dedicated break relief. When a nurse takes a break, someone must cover their patients. If no one is available, the nurse faces a choice: skip the break (violating their rights and potentially state law) or take the break knowing their patients may not receive timely attention. Neither choice is acceptable.

## 2.2 Conflict 2: Low Census Procedures vs. Income Security

Every hospital experiences periods when patient volume drops below staffing levels. The hospital's need to reduce labor costs during these periods conflicts with nurses' need for predictable income.

All ten agreements contain low census provisions allowing shift cancellations or reductions. All ten also contain provisions intended to distribute the burden equitably. The conflict arises because these goals are difficult to reconcile in practice.

The core dilemma: agreements establish rotation systems for fairness, but simultaneously grant management discretion to override rotation based on skill, competence, or operational needs. This discretion, while operationally necessary, can undermine the equity the rotation system was designed to ensure.

[4] RCW 49.12 and WAC 296-126-092 establish meal and rest break requirements for employees. Healthcare workers have specific provisions under WAC 296-126-092(4) allowing for "on duty" meal periods when the nature of work prevents relief, but workers must be compensated and truly free to eat.

| Health System | Manifestation |
|---|---|
| EvergreenHealth | Rotation equity vs. skill-based selection override |
| Overlake | Low census rotation with PTO required |
| Seattle Children's | No Pay Bank with extra shift requirements |
| Providence Sacred Heart | 1.5–2 hour notice cancellations |
| Providence Holy Family | Low census vs. schedule stability |
| Ocean Beach | Two rotation limit then seniority-based |
| Whidbey | Staffing plan adherence vs. flex down authority |
| Skagit Regional | 60-hour cap but nurse must self-declare in real time |
| Grays Harbor | Volunteer counting against rotation discourages volunteering |
| Cascade | Make-whole replacement shifts blocked by rest/OT waivers |

Table 3: Low Census Conflict Manifestations

### 2.3 Conflict 3: Floating Requirements vs. Competency Protections

Hospitals need deployment flexibility to match staffing to patient needs across units. Nurses cannot safely perform tasks outside their competency. All ten agreements allow floating but provide inadequate guidance on competency boundaries, creating ambiguity about when a nurse can reasonably decline an assignment.

| Health System | Manifestation |
|---|---|
| EvergreenHealth | "Staff helper" role creates ambiguity |
| Overlake | Floating required vs. specialized competency limits |
| Seattle Children's | Floating to meet needs vs. specialty boundaries |
| Providence Sacred Heart | "Collaboratively provide care until resolved" |
| Providence Holy Family | Float to meet staffing vs. specialized competencies |
| Ocean Beach | Precepted nurse competency gap |
| Whidbey | Embedded in staffing provisions |
| Skagit Regional | Per diem required skills vs. employer controls availability |
| Grays Harbor | "Adequate orientation" defined by employer |
| Cascade | Cross-training premium requires quarterly outside-unit shift |

Table 4: Floating Conflict Manifestations

The core dilemma: agreements typically state that nurses may float but will not be required to perform tasks outside their competency. However, "competency" is rarely defined objectively. A nurse floated to an unfamiliar unit must decide whether to accept tasks they feel unprepared for (risking patient safety) or decline tasks (risking discipline for insubordination). The agreement provides the right to refuse but not the clarity to exercise it confidently.

### 2.4 Conflict 4: Schedule Stability vs. Operational Changes

Agreements promise schedule stability, typically requiring "mutual consent" for changes. They simultaneously grant management broad scheduling authority for operational needs. This creates ambiguity about when changes require consent and when they can be imposed.

The core dilemma: nurses make life arrangements based on posted schedules. When those schedules

| Health System | Manifestation |
|---|---|
| EvergreenHealth | Within low census provisions |
| Overlake | PTO scheduling vs. operational override |
| Seattle Children's | Mutual consent vs. "emergency conditions" exception |
| Providence Sacred Heart | Posted schedule consent vs. short-notice cancellation |
| Providence Holy Family | Cannot change without express consent vs. emergencies |
| Ocean Beach | Schedule adjustment rights vs. mutual agreement |
| Whidbey | Schedule stability vs. operational needs |
| Skagit Regional | "Mutually agreeable" vs. "shift rotation when necessary" |
| Grays Harbor | No schedule changes without consent vs. staffing holes |
| Cascade | 14-day request deadline vs. unpredictable life events |

Table 5: Schedule Stability Conflict Manifestations

change without consent, it disrupts childcare, second jobs, education, and personal commitments. Yet operational needs are real and unpredictable. The agreements acknowledge both concerns without resolving the tension between them.

# 3 Additional Common Conflicts

Beyond the four universal conflicts, several others appeared in most agreements analyzed.

## 3.1 Near-Universal Conflicts (8–9 of 10 Agreements)

| Conflict | Description | Prevalence |
|---|---|---|
| Mandatory Overtime vs. Fatigue Protections | Overtime limits stated vs. coverage gaps create pressure to extend shifts | 9/10 |
| PTO Access vs. Staffing-Based Denials | Earned time off vs. denial for staffing needs plus forfeiture caps | 9/10 |
| Seniority Rights vs. Skill-Based Discretion | Seniority governs decisions "except as determined by employer" | 8/10 |

Table 6: Near-Universal Conflicts

## 3.2 Common Conflicts (5–7 of 10 Agreements)

| Conflict | Description | Prevalence |
|---|---|---|
| Staffing Committee Authority vs. Management Rights | Committees "responsible for" plans that management has "right to" override | 7/10 |
| Charge Nurse Responsibilities vs. Patient Load | Leadership duties assigned without workload reduction | 6/10 |
| Per Diem Flexibility vs. Minimum Requirements | Flexible status requires specific weekend/holiday commitments | 6/10 |
| Grievance Timelines vs. Access Constraints | Strict deadlines with union business restricted to non-working time | 6/10 |
| Continuing Education vs. Paid Time Access | Required CE with education time subject to staffing approval | 6/10 |
| Overtime Pre-Authorization vs. Clinical Necessity | Must be authorized in advance vs. patient needs require staying | 5/10 |

Table 7: Common Conflicts

# 4 Conflict Distribution by Category

The identified conflicts cluster into recognizable categories, suggesting they arise from fundamental tensions rather than drafting oversights.

| Category | Examples | Prevalence |
|---|---|---|
| Meal/Rest Breaks | Break rights vs. patient care continuity | 10/10 |
| Low Census/Staffing Flex | Income stability vs. flex down | 10/10 |
| Floating/Competency | Float requirements vs. competency limits | 10/10 |
| Schedule Stability | Mutual consent vs. operational changes | 10/10 |
| Overtime/Fatigue | OT limits/rest vs. coverage needs | 9/10 |
| PTO/Leave Access | Earned time vs. usage restrictions | 9/10 |
| Seniority/Discretion | Seniority rights vs. competency override | 8/10 |
| Staffing Committee | HSC/plan authority vs. management rights | 7/10 |
| Per Diem Structure | Flexibility vs. requirements | 6/10 |
| Charge Nurse Role | Leadership duties vs. patient load | 6/10 |
| Grievance/Due Process | Timelines vs. access constraints | 6/10 |
| Education/Training | CE requirements vs. paid access | 6/10 |
| OT Authorization | Pre-approval vs. clinical reality | 5/10 |

Table 8: Conflict Categories and Prevalence

# 5 Analysis

## 5.1 Why These Conflicts Persist

The conflicts identified in this analysis are not new. Many have existed in various forms for decades. Their persistence suggests they arise from genuine tensions that cannot be easily resolved through contract language alone.

Consider the break coverage conflict. Providing uninterrupted breaks requires either overstaffing (expensive) or dedicated break relief positions (also expensive and difficult to fill). The conflict persists not because negotiators failed to notice it, but because resolving it requires resource allocation that neither party has prioritized sufficiently.

Similarly, the low census conflict reflects a genuine tradeoff. Hospitals cannot predict patient volume precisely. Nurses cannot absorb unlimited income variability. Any solution involves costs that someone must bear.

## 5.2 The Common Resolution Gap

Most conflicts share a structural pattern: two legitimate requirements are stated without defining what happens when they conflict. Agreements typically lack:

- Priority rules when requirements clash
- Escalation mechanisms with defined timelines
- Documentation and accountability for exceptions
- Feedback loops to identify recurring problems

This gap creates ambiguity that charge nurses and managers must resolve in real time, often under pressure, without clear guidance. The result is inconsistency: similar situations handled differently depending on who is working, creating perceptions of unfairness even when all parties act in good faith.

## 5.3 Staffing as Root Cause

Many conflicts trace back to insufficient baseline staffing.[6] Break coverage, charge nurse workload, floating, and mandatory overtime all become more acute when units operate at minimum staffing. The agreements attempt to manage consequences of tight staffing rather than prevent the underlying condition.

This observation does not imply that health systems are deliberately understaffing. Financial constraints, labor market conditions, and regulatory requirements all shape staffing decisions.[7] But it suggests that addressing individual conflicts without addressing staffing levels may produce limited results.

[6] The 2024 Washington Center for Nursing Burnout Survey found that 33.1% of Washington nurses report feeling emotionally drained "every day." See Washington Center for Nursing, "Prioritizing Nursing Well Being to Strengthen the Workforce: 2024 Washington Nursing Burnout Survey and Next Steps," July 2024.

[7] National turnover data indicates acute care hospital RN turnover stood at 18.3% in late 2024, with an average replacement cost of $53,600 per nurse. See NSI Nursing Solutions, "2025 NSI National Health Care Retention & RN Staffing Report."

### 5.4 The "Premium Pay Solves It" Fallacy

Multiple agreements attempt to resolve safety-related conflicts by paying premiums. For example, missed breaks trigger overtime pay; working fatigued earns a premium; being floated to an unfamiliar unit may come with extra compensation. While these provisions acknowledge the burden, they do not resolve the underlying conflict.

Paying time-and-a-half for working fatigued does not reduce fatigue risk. Compensating a nurse for a missed break does not provide the rest that break was meant to deliver. Premium pay monetizes the conflict rather than evaporating it.[8] This pattern appeared across multiple agreements and suggests a structural limitation in how negotiations address certain categories of conflict.

### 5.5 Rural and Small Hospital Amplification

Ocean Beach, Whidbey Health, Grays Harbor, and Cascade Medical Center, all smaller facilities serving rural or critical access populations, showed conflict patterns amplified by thinner staffing margins.[9] When every nurse is essential, low census decisions, float assignments, and overtime become higher-stakes. The same contractual ambiguity that creates minor friction at a large urban hospital can create significant operational disruption at a critical access facility.

These facilities also showed unique conflicts around technology assumptions (references to "residential phones" for contact), training maintenance with limited specialty shift availability, and privacy concerns with roster data. The smaller scale both concentrates impact and introduces constraints that larger facilities do not face.

[8] This pattern aligns with what Goldratt termed a "compromise" rather than a "breakthrough" solution. Compromise solutions reduce the pain of a conflict without eliminating its cause. See Goldratt, Eliyahu M., *It's Not Luck*, North River Press, 1994.

[9] Critical access hospitals face particular constraints. Ocean Beach Hospital and Cascade Medical Center operate under federal critical access hospital designation, which limits bed counts and imposes specific staffing considerations.

# 6 Implications

## 6.1 For Labor Relations

The presence of embedded conflicts does not indicate failed negotiations. Both parties typically recognize these tensions and have made reasonable attempts to balance competing interests. However, awareness of the specific conflicts and their operational manifestations may inform future bargaining.

Rather than relitigating broad principles, negotiations might focus on developing resolution mechanisms for specific high-impact conflicts. For example, a joint labor-management working group might address break coverage by piloting dedicated relief positions on selected units, with evaluation criteria agreed in advance.

## 6.2 For Workforce Strategy

In a competitive labor market, differentiation matters. Most health systems in a region offer similar wages, similar benefits, and similar working conditions. A system that demonstrably addressed even one universal conflict would offer something distinctive.

Consider break coverage. A health system that implemented reliable break relief, not as a pilot but as standard practice, would be offering something most competitors do not. This is harder to replicate than a wage increase because it requires operational commitment, not just budget allocation.

The same logic applies to the other universal conflicts. Clear, objective criteria for low census rotation would address fairness concerns. Defined competency matrices for floating would reduce ambiguity and anxiety. These are not easy problems, but solving them creates durable competitive advantage.

## 6.3 For Patient Care

Embedded conflicts affect patient care indirectly but meaningfully. A nurse who skips breaks experiences fatigue. A nurse floated to an unfamiliar unit works more slowly and with less confidence. A nurse anxious about low census may be distracted by financial concerns.

None of these effects are dramatic. But accumulated across shifts, units, and months, they shape the care environment. Resolving embedded conflicts is not just a labor relations issue; it is a quality issue.

# 7 Limitations

This analysis has several limitations that should inform interpretation of the findings.

**Document-based analysis.** The analysis examined contract language, not operational practice. A conflict that exists on paper may be well-managed in practice through informal norms, or may be more severe than the language suggests. Interviews with nurses and managers would provide additional perspective.

**Sample size.** Ten agreements represent a substantial sample of Washington State health systems but not a comprehensive survey. Other agreements may contain conflicts not identified here, or may have resolved conflicts that appear in this sample.

**Point-in-time analysis.** Collective bargaining agreements evolve. The agreements analyzed were current as of late 2025, but provisions may have changed in subsequent negotiations.

**Interpretive methodology.** Identifying "conflicts" requires judgment about what constitutes incompatible requirements. Others reviewing the same documents might reach different conclusions about specific provisions.

**No outcome data.** This analysis does not link identified conflicts to measurable outcomes such as grievance rates, turnover, or patient safety events. Such linkage would strengthen the case for prioritizing particular conflicts but was beyond the scope of this research.

# 8 Conclusion

This analysis identified 134 structural conflicts embedded in collective bargaining agreements between ten Washington State health systems and WSNA. Four conflicts appeared in all ten agreements examined: tensions around meal/rest breaks and continuous patient care, low census and income security, floating requirements and competency protections, and schedule stability versus operational flexibility.

These universal conflicts suggest opportunities. They are not unique to any single health system, so addressing them does not require waiting for industry-wide change. A health system that developed effective resolution mechanisms would differentiate itself in ways that matter to nurses considering employment options.

The findings also suggest caution. These conflicts have persisted because they reflect genuine tensions that cannot be resolved through contract language alone. Meaningful resolution requires operational investment, not just policy revision. Premium pay, while acknowledging burden, does not resolve the underlying dilemmas.

A final observation: the conflicts identified in this analysis may reflect, in part, how health systems conceptualize nursing itself. When nursing is viewed primarily as a labor cost to be minimized, contract negotiations naturally focus on flexibility, control, and cost containment—and the organization tries to minimize hours *spent*. When nursing is viewed as a critical capacity constraint that enables the organization's core mission, the conversation shifts toward protection, investment, and optimization of that scarce resource—and the organization focuses on how hours are *invested*. The same contractual provisions can emerge from either framing, but the operational culture surrounding them differs markedly. Systems that genuinely treat nursing capacity as mission-critical may find that some conflicts resolve themselves—not through better contract language, but through different daily decisions about how that capacity is managed.

For health systems competing for nursing talent in a constrained labor market, the question is whether these conflicts represent intractable features of the employment relationship or addressable problems that have simply not received sufficient attention. This analysis suggests the latter. The conflicts are real, but they are not inevitable.

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## Request for Feedback

This analysis would benefit from additional perspectives.

If you work in one of the health systems analyzed, I would welcome your assessment of whether the identified conflicts match your operational experience. Are there conflicts I missed? Are there conflicts I identified that are well-managed in practice?

If you work in labor relations or nursing leadership at other Washington State health systems, I would be interested to learn whether similar patterns appear in your agreements.

If you see methodological problems with this analysis, or conclusions not supported by the evidence, please let me know.

## Additional Materials Available

The detailed analyses underlying this report, including proposed resolution strategies for each identified conflict, are available from the author upon request. The author also has the capacity to analyze additional collective bargaining agreements using the same methodology.

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# Appendix: Methodology

## Theory of Constraints Conflict Analysis

The Theory of Constraints, developed by Dr. Eliyahu Goldratt, includes a tool called the "Evaporating Cloud" for analyzing conflicts. The tool structures conflicts as follows:

- **A (Common Goal):** What both parties are trying to achieve
- **B and C (Requirements):** Two legitimate needs that must be met to achieve the goal
- **D and D' (Prerequisites):** Actions or conditions required to meet each need
- **The Conflict:** D and D' cannot both be satisfied simultaneously

Resolution involves identifying and challenging the assumptions that make D and D' appear incompatible. Often, the conflict exists because of unstated assumptions about how requirements must be met, not because the requirements themselves are truly incompatible.

## Document Sources

The collective bargaining agreements analyzed were obtained from public sources including WSNA's website and health system publications. All agreements were current as of the analysis date in late 2025.

## Conflict Identification Process

Each agreement was reviewed systematically, section by section. Potential conflicts were flagged when provisions appeared to create situations where compliance with one requirement would necessitate violation of another. Each potential conflict was then analyzed using the Evaporating Cloud structure to confirm that a genuine dilemma existed.

Conflicts were categorized and compared across agreements to identify patterns. Universal conflicts (appearing in all ten agreements) were distinguished from near-universal conflicts (appearing in eight or nine), common conflicts (appearing in five to seven), and unique conflicts (appearing in only one or two agreements).

# References


## Collective Bargaining Agreements Analyzed

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- EvergreenHealth and WSNA, Employment Agreement, 2025–2027.
- Grays Harbor Community Hospital and WSNA, Employment Agreement, 2023–2026.
- Ocean Beach Hospital and WSNA, Employment Agreement, 2024–2027.
- Overlake Hospital Medical Center and WSNA, Employment Agreement, 2024–2027.
- Providence Holy Family Hospital and WSNA, Employment Agreement, 2023–2025.
- Providence Sacred Heart Medical Center and WSNA, Employment Agreement, 2022–2025.
- Seattle Children's Hospital and WSNA, Employment Agreement, 2024–2027.
- Skagit Regional Health and WSNA, Employment Agreement, 2023–2026.
- Whidbey Health Medical Center and WSNA, Employment Agreement, 2024–2026.

All agreements obtained from WSNA union pages at https://www.wsna.org/union or from health system publications.

## Washington State Law

- RCW 49.12: Industrial Welfare (meal and rest break requirements)
- RCW 49.28: Hours of Health Care Facility Employees (mandatory overtime limitations)
- RCW 70.41: Hospital Licensing and Regulation (staffing plan requirements)
- WAC 296-126-092: Meals and Rest Periods
- Senate Bill 5236: "Concerning hospital staffing standards," enacted 2023

## Theory of Constraints

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